Exemestane · daily hormone therapy

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Joints and muscles

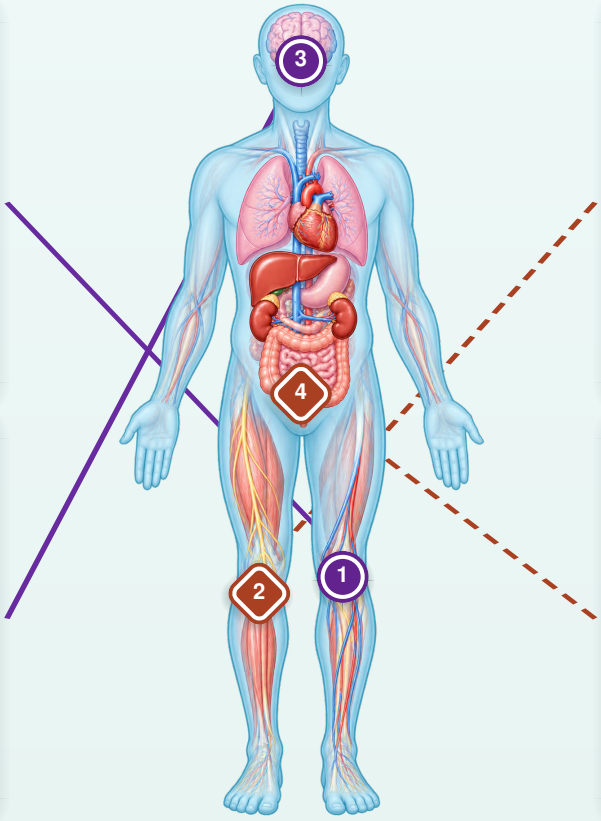
Exemestane

Pain or stiffness can affect activity; report persistent or limiting symptoms.

2 Bone strength

Exemestane

Bone loss may be silent. Your risk assessment and bone-density plan matter.



3 Hot flushes / sleep

Exemestane

Warm flushes and disrupted sleep can be treated; tell the team.

4 Vaginal / sexual symptoms

Exemestane

Dryness or discomfort deserves support. Ask before using hormone-containing treatments.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report persistent limiting joint pain, new severe bone pain or a possible fracture, jaundice or marked new symptoms. Do not wait for the next scheduled review if you are becoming unwell. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take exemestane once daily continuously, preferably after a meal. There is no planned week off. Confirm the prior-treatment and duration plan.

Swallow the tablet whole, preferably after a meal; do not chew or crush it. Skip a missed dose and continue at the next scheduled time; do not double the dose.

Checks and follow-up

full blood count (FBC), renal and liver profile, confirmation of menopause when uncertain, and bone / fracture assessment. Consider baseline vitamin D as specified by the product information. The NICE adjuvant baseline-dual-energy X-ray absorptiometry (bone density) (DXA) recommendation has an exception for patients already receiving adjuvant bisphosphonates. Review symptoms, adherence and bone health. Repeat bone density and lipids when indicated; assess other blood tests according to the clinical plan.

The whole-treatment map distinguishes early-breast follow-up from advanced-disease response assessment.

Support between reviews

Symptoms and daily life

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.

Support medicines

No routine antiemetic, steroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report persistent limiting joint pain, new severe bone pain or a possible fracture, jaundice or marked new symptoms. Do not wait for the next scheduled review if you are becoming unwell.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00376/

Public information only. No personal information is encoded.